

AETHERA · SPECIALTY BILLING SERIES

Psychiatry Billing Solutions

Maximizing Revenue Across Psychotherapy, Medication Management, Telehealth, and Behavioral Health Integration

PREPARED BY Aethera Healthcare Solutions · Lakeland, FL

SCOPE CPT 90791-90838 · 99202-99215 · 90833/90836/90838 · BHI 99484, 99492-99494

UPDATED 2026 Edition · aetherahealthcare.com · +1 (863) 694-0325



BEHAVIORAL HEALTH



ABOUT AETHERA

Maximizing Revenue. Minimizing Burden.

Aethera Healthcare Solutions is your full-service medical billing partner headquartered in Lakeland, FL. We handle coding, claims, payments, denials, and collections — so you can focus on patients.

500+

Practices Served
across 25+ medical specialties

\$12M+

Revenue Recovered
denied & underbilled claims recouped

95%

Clean Claim Rate
vs 84% industry avg

< 30 d

Average AR
days in accounts receivable

01 · DIFFERENTIATOR

Personal, Not Corporate

Dedicated account team with deep specialty/payer knowledge.



02 · DIFFERENTIATOR

Performance Targets, In Writing

96%+ net collection · < 5% denial · 95%+ coding accuracy · 24h posting · 48h submission · 65%+ appeal success.



03 · DIFFERENTIATOR

Zero Risk to Start

No setup fees, no long-term contracts, 90-day cancel, free 5-day audit.





PSYCHIATRY · MARKET CONTEXT

Why Psychiatry Billing Matters Now

Behavioral health demand surged post-pandemic — but psychiatric billing lags behind physical health billing in parity enforcement, telehealth coverage, and integration with primary care. The result: ~\$8B/year in un-billed or under-billed psychiatric services.

\$ 280 B 

US mental health treatment spend (2024)

Includes therapy, medication, inpatient, and outpatient.

57 M 

Americans with mental health condition (2024)

Up from 47M pre-pandemic; ~43% receive any treatment.

~ 32 % 

Psychiatric claim denial rate (industry avg)

Highest of any specialty, driven by parity violations and telehealth coding errors.

\$ 8 B+ 

Annual un-billed psychiatric services (est.)

From add-on codes, BHI, telehealth under-coding, and parity denials.



NET EFFECT

Psychiatric practices that capture every add-on psychotherapy code (**90833/90836/90838**), enroll in BHI (**99484, 99492-99494**), and master telehealth parity billing can recover **12-20% of total revenue** currently lost.



The Four Pillars of Psychiatry Billing

Each pillar represents a distinct service family with unique coding, documentation, and payer requirements.



01 · E/M + THERAPY

Office E/M & Psychotherapy

Diagnostic interview (90791–90792), psychotherapy codes (90832–90837), and E/M with therapy add-on (90833/90836/90838). Add-on codes are the #1 under-billed category.

CPT 90791–90792 · 90832–90837 ·
90833/90836/90838



02 · MED MGMT

Medication Management

E/M visits focused on psychiatric medication management (99202–99215). M0064 for med-mgmt-only (no therapy). Long-acting injection admin (96372).

CPT 99202–99215 · M0064 · 96372
+ J-code



03 · TELEHEALTH

Telehealth Psychiatry

Telehealth psychotherapy and E/M with modifier 95 (video) or GT (audio-only). Place of service 02/10. Parity laws mandate equal reimbursement in 42+ states.

Modifier 95 · POS 02/10 · Mental
health parity



04 · BHI

Behavioral Health Integration

BHI codes for integrated behavioral health in primary care: 99484 (BHI general), 99492–99494 (CoCM tiers). Monthly recurring revenue per patient.

CPT 99484 · 99492–99494 ·
CoCM



PILLAR 01 · E/M + THERAPY

Diagnostic Interviews, Therapy & Add-On Codes

Psychiatric office encounters combine diagnostic interviews, psychotherapy, and E/M medication management. The critical revenue lever is the add-on psychotherapy codes (90833, 90836, 90838) — appended to E/M codes when psychotherapy is provided in the same session. Most psychiatric practices deliver therapy-with-E/M but only bill the E/M code, leaving \$50–100 per encounter on the table.

01 Diagnostic interview

90791 psychiatric diagnostic eval, no medical (\$185); 90792 with medical assessment (\$247). Billed once per episode of care.

02 Psychotherapy codes

90832 30 min (\$75); 90834 45 min (\$105); 90837 60 min (\$145). Timed codes; document start/stop.

03 Add-on codes (highest under-billed)

90833 add 30 min therapy to E/M (\$45); 90836 add 45 min (\$60); 90838 add 60 min (\$80).



A psychiatric practice billing **1,000 E/M visits/year** with 60% including therapy can add **~\$50K/year** by appending **90833** to every applicable E/M.

CPT Quick Reference

2026 NAT'L AVG

Code	Service	Time	Avg \$
90792	Psych diagnostic eval w/ medical	60 min	\$247
99213	Est. E/M, low MDM	30–39 min	\$92
99214	Est. E/M, moderate MDM	40–54 min	\$131
90833	+30 min therapy add-on	+30 min	\$45
90834	Psychotherapy, 45 min	45 min	\$105
90837	Psychotherapy, 60 min	60 min	\$145

Highlighted row = highest under-billed code in psychiatry.

PILLAR 02 · MEDICATION MGMT

Psychiatric Medication Management & Injections

Medication management visits use standard E/M codes (99202–99215) when therapy is also provided, OR the simpler M0064 code for med-mgmt-only encounters (no therapy). Long-acting injectable (LAI) antipsychotics require both administration code 96372 and drug J-code.

01 · E/M WITH MED MGMT

E/M with Medication Management

99213–99215 based on MDM complexity. Most psych med-mgmt visits are 99213–99214.

Modifier 25 if same-day as therapy (separately identifiable).

02 · MED-MGMT ONLY

M0064 (Med-Mgmt Only)

Simplified code for psychiatric medication management without therapy. ~\$75 reimbursement. Use when visit is purely med review/adjustment, no significant therapy.

03 · LAI ADMINISTRATION

Long-Acting Injectable (LAI) Administration

96372 therapeutic injection SC/IM (\$35). Paired with drug J-code: J352 aripiprazole (\$1,500), J2794 risperidone (\$630), J3315 paliperidone (\$1,200).

04 · GENETIC TESTING

Genetic Testing for Psychiatric Meds

CPT 0204U, 0205U (proprietary lab analyses). Used for pharmacogenomic testing when standard SSRIs/SNRIs fail. ~\$500–2,000 reimbursement.

MODIFIER 25

Modifier 25 for E/M + Same-Day Therapy

When billing E/M (99202–99215) AND psychotherapy (90832–90837) on same day, append **modifier 25** to E/M and bill both codes. Cannot use 90792 + 90832 same day.

LAI VS ORAL

LAIs vs Daily Oral Meds

LAIs (\$1,500/injection x monthly) generate significant drug revenue but require prior auth and 30-day cycle tracking. Adherence is dramatically higher with LAIs.

PHARMACOGENOMICS

Pharmacogenomic Testing

Coverage expanding; Cigna/Aetna cover for ≥ 2 failed medication trials. Documentation of failed trials required for PA.



PILLAR 03 · TELEHEALTH

Video & Audio-Only Behavioral Health

Telehealth is now the dominant modality for psychiatric care — ~ 60% of psychiatric visits post-pandemic are telehealth. Federal mental health parity laws and the Consolidated Appropriations Act of 2023 require equal reimbursement for in-person and telehealth psychiatric services. But correct modifier use (95 vs GT) and place-of-service (02 vs 10) determine whether the claim pays at all.

01 Modifier 95 VIDEO

Synchronous audio-video telehealth. POS 02 (telehealth provided from distant site). Reimbursed at parity with in-person under most payer policies.

02 Modifier GT / F3 AUDIO-ONLY

Audio-only telehealth (phone call). Allowed for behavioral health only under CAA 2023 extension. POS 10 (telehealth provided from patient home).

03 Cross-state licensure

Provider must be licensed in patient's state at time of service. PSYPACT (psychology interstate compact) and impending psychiatry compacts ease this; verify before each visit.



Telehealth psychiatric visits reimbursed at parity = \$92-145 per visit. Audio-only allowed when video unavailable, but must be documented as such.

~ 60%

Psychiatric visits via telehealth

2024 share of total psychiatric encounters

42+

States mandate mental health parity

In-person = telehealth reimbursement

POS 02

Telehealth distant site

Provider location for video visits

POS 10

Telehealth patient home

Audio-only BH allowed

♦ PILLAR 04 · BHI

Integrated Behavioral Health in Primary Care

BHI codes reimburse behavioral health care management services delivered by primary care practices in collaboration with psychiatric consultants. Two tracks: BHI General (99484) and Collaborative Care Model (CoCM, 99492–99494). Both generate monthly recurring revenue per enrolled patient.

01 BHI General (99484)

Behavioral health care management, ≥20 min/month clinical staff time + psychiatric consultation. Monthly billing. ~\$50/month per patient. Lower complexity than CoCM.

CPT 99484

02 CoCM Tier 1 (99492)

Initial month of CoCM: ≥70 min of clinical staff time + psychiatric consultant + 14 min per month. First month only. ~\$160/month.

CPT 99492 · CoCM

03 CoCM Subsequent (99494)

Subsequent months CoCM: ≥60 min/month clinical staff time + psychiatric consult. Monthly billing after first month. ~\$110/month per patient.

CPT 99494 · CoCM

04 CoCM Add-on (99493)

Add'l 30 min of CoCM time beyond 99494 base. Use when complex patient requires >60 min/month. ~\$50/month add-on.

CPT 99493 · Add-on

\$50 /mo

BHI general per patient (99484)

\$160 /mo

CoCM first month (99492)

\$110 /mo

CoCM subsequent (99494)

~ \$1,260 /yr

Annual recurring revenue per CoCM patient (Year 2+)



COCM IMPLEMENTATION REQUIREMENTS

(1) Behavioral health care manager, (2) psychiatric consultant (off-site OK), (3) registry/tracking system, (4) measurement-based care (PHQ-9 weekly). Aethera helps implement all 4 components.



Top CPT Codes & 2026 National Average Reimbursement

14

CODES ACROSS 4 PILLARS

Most-billed codes across the four psychiatric pillars. Telehealth variants use modifier 95/GT + POS 02/10.

Code	Service	Time	Modifier	Avg \$
90791	Psych diagnostic eval, no medical	60 min	—	\$185
90792	Psych diagnostic eval w/ medical	60 min	—	\$247
99213	Est. E/M, low MDM	30-39 min	25 if +therapy	\$92
99214	Est. E/M, moderate MDM	40-54 min	25 if +therapy	\$131
90833	+30 min therapy add-on (to E/M)	+30 min	append to E/M	\$45
90834	Psychotherapy, 45 min	45 min	95/GT if TH	\$105
90837	Psychotherapy, 60 min	60 min	95/GT if TH	\$145
M0064	Med-mgmt only, no therapy	brief	—	\$75
96372	Therapeutic injection, SC/IM	per inj	—	\$35
J2352	Aripiprazole LAI (Abilify Maintena)	per dose	—	\$1,500
J3315	Paliperidone LAI (Invega Sustenna)	per dose	—	\$1,200

MODIFIER KEY

Modifier 25 — significant, separately identifiable E/M on same day as psychotherapy.
Modifier 95 — synchronous audio-video telehealth. **Modifier GT** — audio-only telehealth (PH only). CoCM subsequent, 200 min

PLACE OF SERVICE & ABBREVIATIONS

POS 02 — telehealth provided from distant site. **POS 10** — telehealth provided from patient's home (audio-only BH allowed). CoCM = Collaborative Care Model. BHI = Behavioral Health Integration.

Top 8 Psychiatric Denial Reasons & How Aethera Prevents Each

28–32% \$40–120
INDUSTRY DENIAL RATECOST PER REWORK

Psychiatric denial rates average 28–32% — highest of any medical specialty. Each denied claim costs \$40–120 to rework.

RED 22% Telehealth modifier/POS wrong

Root cause: Modifier 95 vs GT confused; POS 02 vs 10 wrong; claim auto-denied.

AETHERA: telehealth rules engine auto-selects correct modifier + POS based on service type.

RED 18% Mental health parity violation

Root cause: Payer reimburses telehealth at lower rate than in-person, violating federal parity law.

AETHERA: parity audit + appeals process invokes Mental Health Parity Act.

AMBER 14% Add-on code missing

Root cause: E/M visit included therapy but 90833/90836/90838 not appended.

AETHERA: add-on code trigger prompts biller when therapy documentation present.

AMBER 11% 90791 + 90832 same day

Root cause: Diagnostic eval + psychotherapy same session billed separately; should be 90792 only.

AETHERA: bundling rules block concurrent diagnostic + therapy billing.

AMBER 9% LAIs prior auth missing

Root cause: Long-acting injectable administered without PA on file; \$1,200+ denied.

AETHERA: PA tracker auto-submits before injection scheduling.

GREEN 7% CoCM registry documentation incomplete

Root cause: 99492–99494 billed without PHQ-9 score in registry.

AETHERA: CoCM registry integration verifies measurement-based care before billing.

GREEN 6% Telehealth cross-state licensure

Root cause: Provider not licensed in patient's state at time of service.

AETHERA: licensure verification at scheduling blocks cross-state telehealth.

GREEN 5% Timed code documentation missing

Root cause: 90832/90834/90837 billed without start/stop time in note.

AETHERA: timed-code documentation checklist before claim release.

📌 **Cumulative impact:** Aethera reduces psychiatric denial rate from industry avg **30%** to **< 10%** — recovering ~ **\$180K/year** per \$1M practice revenue.

■ CASE STUDY · INTEGRATED PSYCHIATRIC RCM

4-Psychiatrist Practice — Annual Revenue Impact

\$2.8M **800/mo** **80**
ANNUAL REVENUE VISITS · 60% LAI PATIENTS
TH

A \$2.8M-revenue practice models the combined upside of add-on code capture, telehealth parity enforcement, and LAI prior auth recovery.

+ COLUMN 01 Add-On Code Capture

- 9,600 annual E/M visits
- 60% include therapy (5,760 visits)
- Pre-Aethera add-on billing: 35% (2,016)
- Post-Aethera: 85% (4,896)
- Recovered: 2,880 add-ons × \$45 avg

ANNUAL REVENUE
\$129,600

▶ COLUMN 02 Telehealth Parity Enforcement

- 5,760 annual telehealth visits
- Pre-Aethera parity denial rate: 18%
- Post-Aethera: 4%
- Recovered: 806 visits × \$92 avg
- —

ANNUAL REVENUE
\$74,152

+ COLUMN 03 LAI Prior Auth Recovery

- 80 LAI patients, monthly injections
- Pre-Aethera PA denial rate: 22%
- Post-Aethera PA denial rate: 6%
- Recovered injections: 192/year × \$1,200 avg
- —

ANNUAL REVENUE
\$230,400

■ **Combined annual revenue recovery: \$434,152.** Implementation cost (year 1): ~\$168K (Aethera RCM fees + EHR telehealth integration + registry setup).

8286,152
YEAR-1 NET

158%
ROI



Where OIG, CMS, and State Medicaid Audit First

Top compliance risks across the four psychiatric pillars. Quarterly self-audits recommended.

\$180M +

CMS BH RECOVERIES
SINCE 2022

0 E/M + Therapy

RED

90833 misuse — appending add-on code when therapy time is <30 min or not documented in note; OIG Work Plan 2025-2026.

AMBER

90791 + 90832 same day — diagnostic eval and therapy billed separately on same session; should be 90792 only.

GREEN

Timed code documentation missing — 90832/90834/90837 without start/stop time in chart note.

0 Medication Management

RED

LAIs without prior auth — administering \$1,200+ injectable without PA on file; full denial + clawback risk.

AMBER

M0064 misuse — using simplified code when visit complexity actually warrants 99213-99215; upcoding audit risk.

LOW

Pharmacogenomic testing without documented failed trials — payers require ≥2 failed medication trials before covering.

0 Telehealth

HIGH

Cross-state licensure violation — provider not licensed in patient's state at time of telehealth service; criminal exposure in some states.

MEDIUM

Audio-only billed as video — using modifier 95 (video) when service was audio-only (should be GT); automatic denial on audit.

LOW

Telehealth consent not documented — federal/state laws require documented patient consent for telehealth services.

0 Behavioral Health Integration

RED

CoCM without psychiatric consultant — billing 99492-99494 without documented psychiatric consultant involvement in care; OIG Work Plan 2025-2026.

AMBER

Registry documentation gaps — CoCM billing requires PHQ-9 score in registry; missing scores trigger audit denial.

LOW

Time threshold fraud — billing 99494 (60+ min) when actual staff time was <60 min; time log required.



OIG Work Plan 2025-2026 explicitly lists psychiatric telehealth parity violations and CoCM billing fraud as active audit targets. **CMS recovered >\$180M** from behavioral health practices since 2022.

Three Actions to Capture Psychiatric Revenue

For psychiatry practice administrators and behavioral health directors leading the next 90 days.

01



Audit add-on code capture

90-DAY ACTION

Pull last 200 E/M visits with documented therapy in chart. Calculate % where 90833/90836/90838 was actually billed. **Target: 85%+ capture rate** (industry avg 35%). Each missed add-on = \$45-80 lost per visit. A 1,000-visit practice adds ~ \$45K/year.

02



Enforce telehealth parity appeals

90-DAY ACTION

Audit last 100 telehealth claims for parity violations (telehealth reimbursed at < 100% of in-person rate). File formal parity appeals citing **Mental Health Parity and Addiction Equity Act + CAA 2023**. Recover 60-80% of underpaid claims.

03



Launch a BHI / CoCM program

90-DAY ACTION

Identify primary care partners. Stand up CoCM infrastructure: behavioral health care manager + psychiatric consultant + PHQ-9 registry. Enroll 100 patients in year 1. **Revenue: \$110K/year of recurring monthly billing** from CoCM codes 99492-99494.

Questions & Discussion



Optimizing Psychiatric Revenue Across E/M, Telehealth, BHI & Med Management



support@aetherahealthcare.com



aetherahealthcare.com



+1 (863)
694-0325